

AMENDED PACKAGE INSERT

SUNITON TABLET

Each tablet contains:-
Orphenadrine Citrate 35mg
Paracetamol 450mg

Product Description:

Round, flat, white, plain tablet with single score on one side only.

Pharmacodynamics:

Orphenadrine citrate is a centrally acting (brain stem) compound which in animals selectively blocks facilitatory functions of the reticular formation. It does not produce myoneural block, nor does it affect crossed extensor reflexes. Orphenadrine prevents nicotine-induced convulsions but not those produced by strychnine. It acts centrally, rather than directly on skeletal muscle, to relax and inhibit spasm of the striated muscle. It does not directly relax tense muscle in man. Orphenadrine also exerts mild anticholinergic effect, of lesser intensity than those of its congener diphenhydramine. It does not cause sedation. Paracetamol produces anti-pyresia through action on the hypothalamic heat-regulating center and analgesia by elevation of the pain threshold.

Pharmacokinetics:

Orphenadrine is readily absorbed from the gastrointestinal tract and is almost completely metabolised to at least 8 metabolites. It is mainly excreted in the urine as metabolites and unchanged drug. A half-life of 14 hours has been reported. Paracetamol is readily absorbed from the gastro-intestinal tract with peak plasma concentration occurring about 30 minutes to 2 hours after ingestion. It is metabolised in the liver and excreted in the urine as the glucuronide and sulphate conjugates. Less than 5% is excreted as unchanged paracetamol. The elimination half life varies from about 1 to 4 hours. Plasma protein binding is negligible at usual therapeutic concentrations but increases with increasing concentrations. It is reported that the binding of paracetamol to plasma protein did not occur with plasma concentration of less than 60ug per ml, corresponding to the usual therapeutic concentration. After toxic doses of paracetamol, up to 45% could be bound to plasma proteins.

Indication:

It is indicated for the relief of painful muscular conditions and tension headache.

Recommended Dosage:

One to two tablets, three times daily.

Route of Administration:

Oral

Contraindications:

Orphenadrine citrate has mild anticholinergic action and is contraindicated in patients with glaucoma, pyloric or duodenal obstruction, stenosing peptic ulcer, prostatic hypertrophy or obstruction at the bladder neck and myasthenia gravis.

It is contraindicated in patients who are hypersensitive to it and also in patients with liver impairment.

Warnings and Precautions:

Orphenadrine citrate should be used with caution in patients with tachycardia, cardiac decompensation, coronary insufficiency or cardiac arrhythmias.

As with any drug, patients should be advised against the ingestion of alcohol during treatment.

It should be given with care to patients with impaired kidney or liver function.

Paracetamol could interfere with paper chromatographic estimations of urinary amino acids by producing drug spots and could interfere technically with laboratory estimations for meta-adrenaline in the urine to produce erroneous raised results.

Paracetamol might interfere with the measurement of serum uric acid concentration when the reagent used was phosphotungstic acid.

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

This preparation contains PARACETAMOL. Do not take any other medicines containing PARACETAMOL at the same time.

Interactions with Other Medicaments:

Orphenadrine has been shown to induce enzymatic systems involving the metabolism of aminopyrine, steroidal contraceptives, griseofulvin, hexobarbital and phenylbutazone. Orphenadrine will potentiate other anticholinergic agents.

Paracetamol should also be given with care to patients taking other drugs that affect the liver. Paracetamol hepatotoxicity has been reported to be enhanced by phenobarbitone, alcohol and doxorubicin.

Pregnancy and Lactation:

Orphenadrine has been taken by only a limited number of pregnant women and women of childbearing age, without an increase in the frequency of malformations or other direct or indirect harmful effects on the human fetus having been observed.

Since safety of the use of this preparation in pregnancy and during lactation has not been established, use of the drug in such patients requires that the potential benefits of the drug be weighed against its possible hazard to the mother and child.

Side Effects:

Adverse effects are mainly due to the mild anticholinergic action of orphenadrine, and are usually associated with higher dosage. Dryness of the mouth is usually the first adverse effect to appear. When the daily dose is increased, possible adverse effects include: tachycardia, palpitation, urinary hesitancy or retention, blurred vision, dilatation of pupils, increased ocular tension, weakness, nausea, vomiting, headache, dizziness, constipation, drowsiness, hypersensitivity reactions, pruritus, hallucinations, agitation, tremor, gastric irritation, and rarely urticaria and other dermatoses.

Side-effects of paracetamol are usually mild, though haematological reactions such as agranulocytosis, thrombocytopenia and pancytopenia have been reported. Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/ Toxic Epidermal Necrolysis have been reported.

Other allergic reactions occur occasionally.

Symptoms and Treatment of Overdose:

Symptoms of orphenadrine overdose are excitement, confusion and delirium leading to coma. Convulsions and tachycardia with dilated pupils and urinary retention may occur.

Paracetamol overdose may cause acute liver damage but symptoms may not appear for up to several days after ingestion.

Gastric lavage should be carried out immediately, regardless of the estimated ingested dose. Convulsions and delirium respond to relatively large doses of diazepam, preferably by mouth. Adequate hydration of the patient is important. It is recommended that the patient be referred to a hospital where early and regular monitoring of plasma paracetamol levels can be carried out. If instituted sufficiently early, treatment with N-acetylcysteine, L-methionine, L-cysteine will minimise liver damage.

Effects on Ability to Drive and Use Machine:

Some patients may experience transient episodes of light-headedness, dizziness or syncope, which may impair their ability to engage in potentially hazardous activities such as operating machinery or driving a motor vehicle.

Storage Conditions: Store below 30°C. Protect from light.

Pack size: Blister pack: 10 x 10, 50 x 10 and 100 x 10 tablets per strip.

Pack size (export only): Bottle pack: A bottle of 1000 tablets.

Shelf -life: 3 years.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:

Sunward Pharmaceutical Sdn. Bhd.

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